

CARDIOLOGY FELLOWS TEACHING SESSION

Lipid Management

The 2026 ACC/AHA Dyslipidemia Guideline

Risk assessment, LDL-C goals, add-on therapy and special populations

[Presenter name · date]

What is new in 2026

1

PREVENT replaces PCE

New risk equations, with 10-year and 30-year risk. The old model overestimated risk by about twofold.

2

LDL-C goals return

Explicit LDL-C and non-HDL-C goals by risk tier, plus a target percent reduction.

3

More biomarkers

Measure Lp(a) at least once. Use apoB selectively. CAC refines risk in selected adults.

4

Earlier action

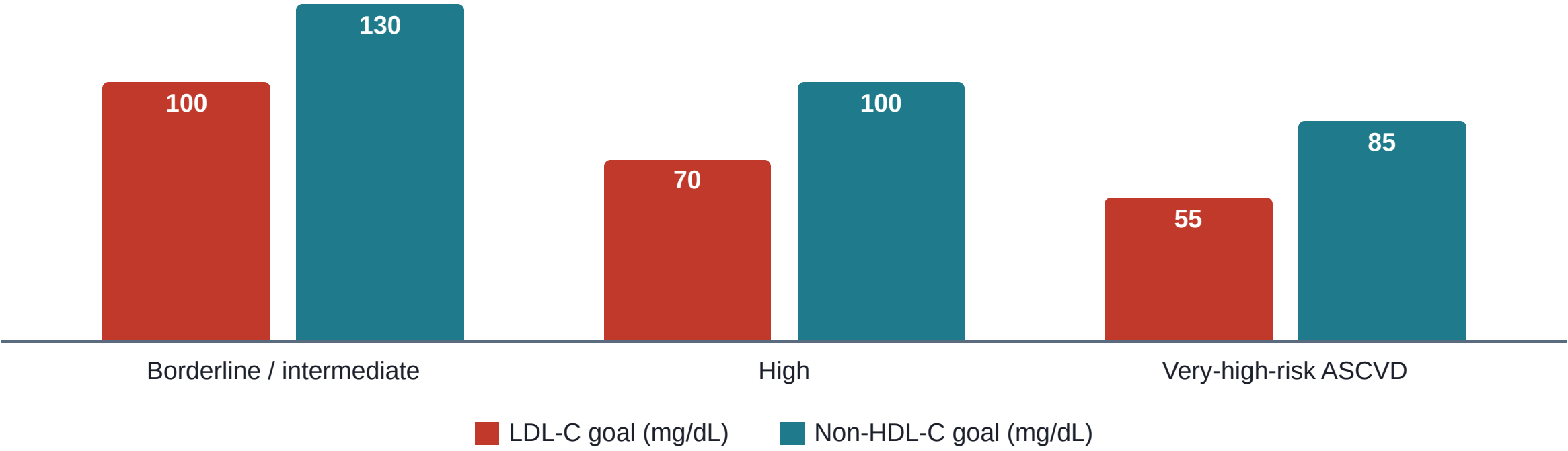
30-year risk and young-adult thresholds support treatment before midlife.

Risk tier decides the LDL-C goal

Risk tier	10-year PREVENT risk	LDL-C goal	Non-HDL-C goal	LDL-C reduction
Borderline	3% to under 5%	< 100 mg/dL	< 130 mg/dL	30 to 49%
Intermediate	5% to under 10%	< 100 mg/dL	< 130 mg/dL	30 to 49%
High	10% or higher	< 70 mg/dL	< 100 mg/dL	At least 50%
Very-high-risk ASCVD	Secondary prevention	< 55 mg/dL	< 85 mg/dL	Per goal

Under 3% is low risk. Values are the targets stated in guideline summaries; verify details in the full text.

Goals step down as risk rises



Refine risk before deciding



PREVENT 10-year

Start here for adults.
Gives borderline,
intermediate or high risk.



30-year risk

Ages 30 to 59. Statin may
be considered if 10-year
risk is under 3%, 30-year
risk is 10% or higher and
LDL-C is 160 to 189
mg/dL.



CAC score

Men 40 and older, women
45 and older. Improves
risk assessment and helps
set goals.



Lp(a) and apoB

Lp(a) once in every adult.
apoB selectively, for
example with high
triglycerides.

Who is treated regardless of LDL-C

Ages 40 to 75

- Diabetes
- CKD stage 3 or 4
- HIV
- LDL-lowering therapy recommended regardless of LDL-C level

Familial hypercholesterolemia

- Consider drug therapy early, even in youth
- CAC of 0 does not justify statin deferral
- Young adults with LDL-C 160 or higher or strong family history: consider therapy

Elevated Lp(a)

- Treat as a risk enhancer
- Intensify LDL-C lowering
- Manage other risk factors more aggressively

Secondary prevention: statin first, then add on

1. Maximally tolerated statin

High-intensity when tolerated.
Recheck lipids and adherence.



2. Above goal? Add on

Ezetimibe and/or a PCSK9 monoclonal antibody, chosen by LDL-C lowering needed and patient preference.



3. Goal reached

LDL-C under 55 mg/dL and non-HDL-C under 85 mg/dL in very-high-risk ASCVD.

LDL-C above 190 mg/dL with clinical ASCVD: same goal (under 55 mg/dL). If needed, add ezetimibe, a PCSK9 monoclonal antibody, inclisiran and/or bempedoic acid.

Lipoprotein(a): measure once

≥ 125 nmol/L

About 50 mg/dL

Risk-enhancing factor. About 1.4-fold higher ASCVD risk.

≥ 250 nmol/L

About 100 mg/dL

At least 2-fold higher estimated ASCVD risk.

Action: measure Lp(a) at least once, then intensify LDL-C lowering and risk-factor control if elevated.

Children, pregnancy and triglycerides

9-11

Screen children

Lipid profile in children 9 to 11 years not previously tested, to find FH and other significant lipid disorders.

8+

Treat FH in youth

LDL-C persistently 160 or higher with an FH picture and inadequate response after 3 to 6 months of lifestyle: start a statin, add others if needed.

Preg

Pregnancy

Not at high ASCVD risk: stop the statin 1 to 2 months before trying to conceive, or when pregnancy is discovered.

TG

High triglycerides

Statin remains the foundation of drug therapy, together with lifestyle change.

Case for discussion

Illustrative case

A 58-year-old man had an acute coronary syndrome 6 months ago. He takes atorvastatin 80 mg daily with good adherence. Fasting LDL-C is 82 mg/dL and non-HDL-C is 112 mg/dL. Lp(a) has never been measured.

What is his goal, and what is your next step?

Answer

- Very-high-risk ASCVD
- Goal: LDL-C under 55, non-HDL-C under 85
- Add ezetimibe and/or a PCSK9 monoclonal antibody
- Measure Lp(a) once

Take-home points

- 1 Use PREVENT for 10-year and 30-year risk, not the old pooled cohort equations.
- 2 Treat to a goal: under 100, under 70, or under 55 mg/dL by risk tier.
- 3 Refine with CAC, Lp(a) once, and apoB when triglycerides are high.
- 4 In very-high-risk ASCVD, add ezetimibe and/or a PCSK9 antibody when above goal.
- 5 Act early in FH, diabetes, CKD, HIV, and in young adults with high lifetime risk.

Source and caveats

Primary source: 2026 ACC/AHA/AACVPR/ABC/ACPM/ADA/AGS/APhA/ASPC/NLA/PCNA Guideline on the Management of Dyslipidemia. J Am Coll Cardiol 2026. doi:10.1016/j.jacc.2025.11.016

Caveats: This deck was prepared from published guideline summaries, not the full text. Check class of recommendation, evidence level, statin intensity and drug-specific details in the original before clinical use or publication.